



Dementia Care

This module is designed for staff in both residential aged care and home care environments, specifically those providing care for clients living with dementia. It aims to provide healthcare staff, regardless of experience level, with a comprehensive understanding of dementia, its various symptoms, and strategies for effectively managing behavioural changes.



Learning Outcomes

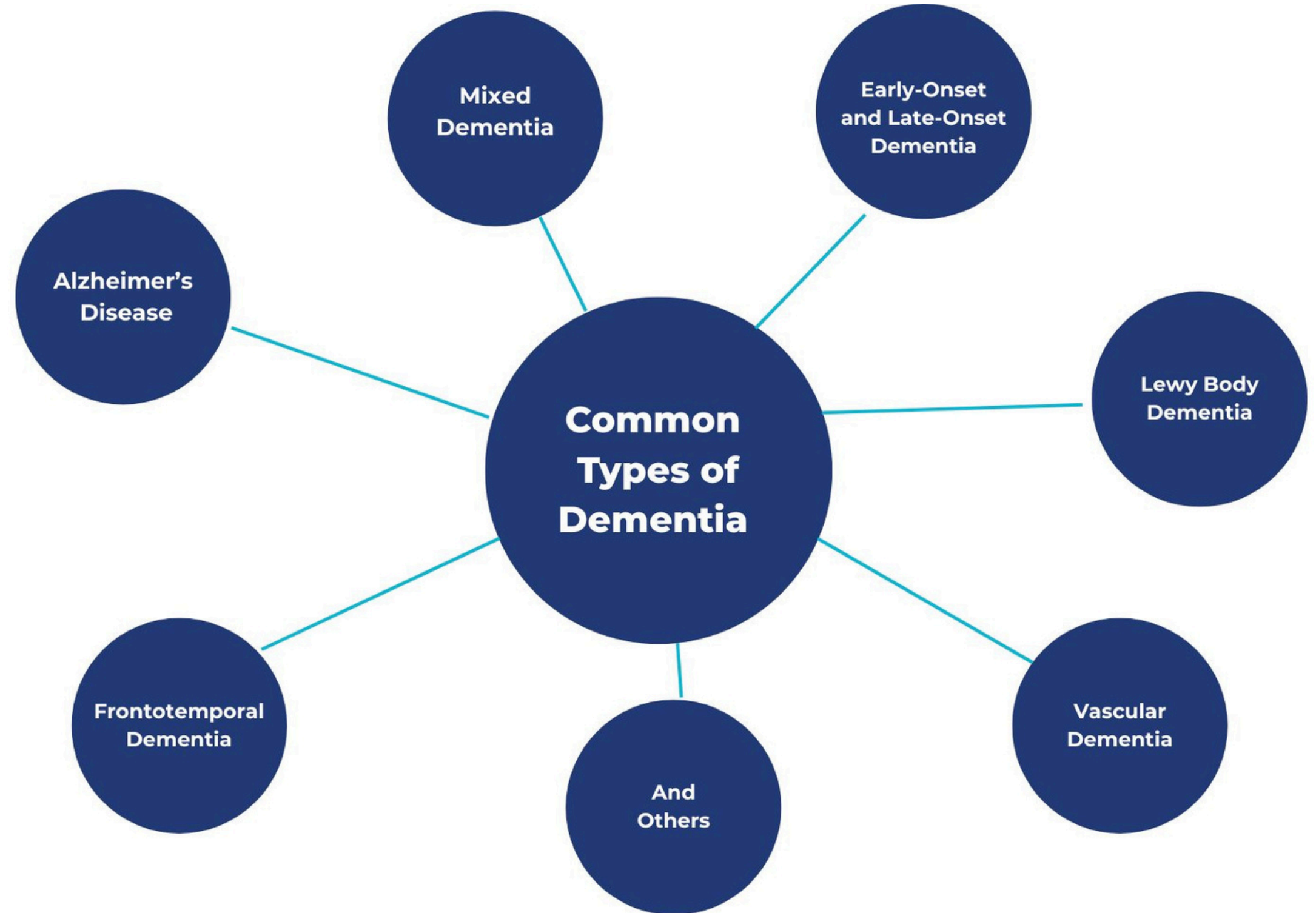
By the end of this training, you will be able to:

- ✓ Explain what Dementia is
- ✓ Recognise symptoms of Dementia in clients
- ✓ Understand common psychological and behavioral symptoms
- ✓ Manage behavioural changes
- ✓ Support individuals with Dementia



What is Dementia?

- Dementia is **not** a single condition.
- It is a term used for several diseases that affect a person's memory, cognitive abilities and behaviour.



Symptoms of Dementia:

Cognitive

- **Memory Loss:** this can include not being able to create new memories, taking longer to retrieve information, or not being able to retrieve information.
- **Impaired coordination & motor function:** this can manifest as difficulties with walking, balance, fine motor skills, and overall movement coordination.
- **Confusion and disorientation:** ranging from mild confusion to more severe difficulties with orientation, memory, and judgment.
- **Difficulty performing tasks, communicating or problem solving.**

Remember:

The term 'dementia' refers to a collection of progressive diseases that present as a multitude of different cognitive psychological and behavioural symptoms.

Dementia is not a singular condition. No two people will experience dementia in exactly the same way.



Symptoms of Dementia:

Psychological

- Personality changes
- Depression
- Anxiety
- Inappropriate behaviour and lack of inhibitions
- Paranoia
- Agitation
- Hallucinations

Remember:

The term 'dementia' refers to a collection of progressive diseases that present as a multitude of different cognitive psychological and behavioural symptoms.

Dementia is not a singular condition. No two people will experience dementia in exactly the same way.



Symptoms of Dementia:

Behavioural

- **Resistance:** reluctance or refusal to participate in or accept help.
- **Aggression:** verbal or physical outbursts, often triggered by confusion, anxiety, or unmet needs.
- **Agitation:** can manifest as increased physical activity like pacing or fidgeting, or as verbal outbursts.
- **Anxiety:** can present as repetitive behaviours, physical symptoms, fast heartbeats or shortness of breath, restlessness, or irritability.
- **Wandering:** individuals with cognitive impairment may leave their home or familiar areas without a clear destination or intention.

Remember:

The term 'dementia' refers to a collection of progressive diseases that present as a multitude of different cognitive psychological and behavioural symptoms.

Dementia is not a singular condition. No two people will experience dementia in exactly the same way.



How Dementia Presents?

Behavioural

- These behaviours may be caused by dementia-related changes in the brain.
- Can also be a response to changes in a person's environment, health or medication.
- It can often be difficult to know what the exact cause is, especially if the person has difficulty communicating.

Remember:

The term 'dementia' refers to a collection of progressive diseases that present as a multitude of different cognitive psychological and behavioural symptoms.

Dementia is not a singular condition. No two people will experience dementia in exactly the same way.





Christine Bryden: A conversation about dementia



Share



Watch on  YouTube



Managing Behavioural and Psychological Symptoms of Dementia (BPSD)

- **Behaviours and emotions are often a direct result of the underlying dementia condition.**
They are not intentional but need a thoughtful, patient approach.
- **BPSD can be stressful, uncomfortable, upsetting and even frightening.**
However, it is important to remember that a client's BPSD are not deliberate or personal actions are often directed at whoever happens to be closest to the client at the time.
- **It's important to understand that displays of BPSD are generally in response to a certain trigger.**
The person is often frustrated or unable to communicate their needs.

There is no one-size-fits-all solution:

Management and support that might work for one person may not always work for another.



Approaching BPSD

- **Calm Environment:** Quiet, familiar surroundings, less confusion.
- **Routine & Consistency:** A predictable schedule reduces anxiety.
- **Non-confrontational Communication:** speak calmly, avoid sudden physical contact.
- **Patience is Key:** don't reprimand; approach with empathy.
- **Remember:** these symptoms are often a direct result of the underlying dementia condition.

There is no one-size-fits-all solution:

Management and support that might work for one person may not always work for another.



Managing Specific Behaviours:

Restlessness & Agitation



Causes

- **Physical changes in the brain:** structural changes in the brain can make it difficult to process information.
- **Environmental factors:** changes in surroundings, sensory overload, and lack of stimulation.
- **Unmet needs:** sleep disturbances, pain & discomfort, social isolation, hunger, thirst, or the need to use the bathroom.



What to Do:

- **Reassure the person**, make them feel safe.
- **Address their discomfort** (hunger, pain, or fatigue)
- **Try calming activities** (music, sensory objects)



Managing Specific Behaviours:

Aggression



Causes

- **Fear:** stemming from memory loss and confusion. **Confusion:** crowded or noisy environments, altered judgment and impulses, and medication. **Frustration:** from communication difficulties, or pain and discomfort.



What to Do:

- **Use simple instructions:** redirect to a calming task. **Remain calm:** speak softly, acknowledge their feelings. **Use “therapeutic lying” if needed:** non-confrontational responses.



Managing Specific Behaviours:

Hallucinations and Delusions



Causes

- **Poor eyesight or hearing.**
- **Physical illness:** including infection, fever, constipation, anaemia, respiratory disease and dehydration.
- **Mental health conditions:** such as schizophrenia.
- **New or changed environments, caregivers or routines.**
- **Feeling overwhelmed:** too many things happening at once.



What to Do:

- **Stay calm and acknowledge feelings.**
- **Use distraction:** soothing activities or conversations.
- **Adjust the environment:** lighting, and noise reduction.
- **If they aren't in distress:** simply observe without intervening.



The ABC Approach to Behaviour Management:

A simple framework to understand and manage behaviours

- **Antecedent:** Identify triggers (environmental or internal).
- **Behaviour:** Document specific actions (e.g. aggression, or resistance).
- **Consequences:** Record responses, note if they escalated or de-escalated the behaviour.
- **Debrief and Decide:** Collaborate to manage the situation.

Remember:

While often distressing, behavioural changes in a person with dementia are **not intended** to hurt you. Often, there is an underlying **trigger** that is causing these behaviours to manifest.

Explore what the underlying causes of changed behaviour might be and address these in an **empathetic way**.

Get to know your clients and learn to **understand their triggers** as well as what makes them feel calm and safe.



Strategies for Communicating with Someone who has Dementia



Effective Communication: Use simple, clear language. Ensure aids (glasses, hearing aids) are available.



Certain behaviours can create frustration and escalate stress for people with dementia.



Loud noises, TV, and crowded spaces can worsen confusion and anxiety in dementia clients, hindering communication.



It's essential to use a calm and respectful tone. Speak slowly and at a moderate pace.



Listen actively and patiently, not just to the words but to the emotions behind them.

Strategies for Communicating with Someone who has Dementia



- ✓ **Engage in Activities:** Familiar or new activities based on abilities.
- ✓ **Promote Physical Activity:** Simple movements or exercises reduce anxiety.
- ✓ **Sensory Stimulation:** Comforting activities (hair brushing, flowers, pets).



Food, nutrition and dining webinar - Supporting people with dementia to eat and ...



Welcome

Supporting people living with dementia to eat and drink well

Panellists



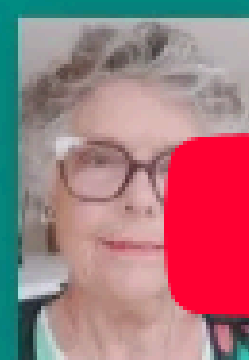
Jessica Zilujko
Director
Food Nutrition and
Dining Advisory Support
Unit
ACQSC



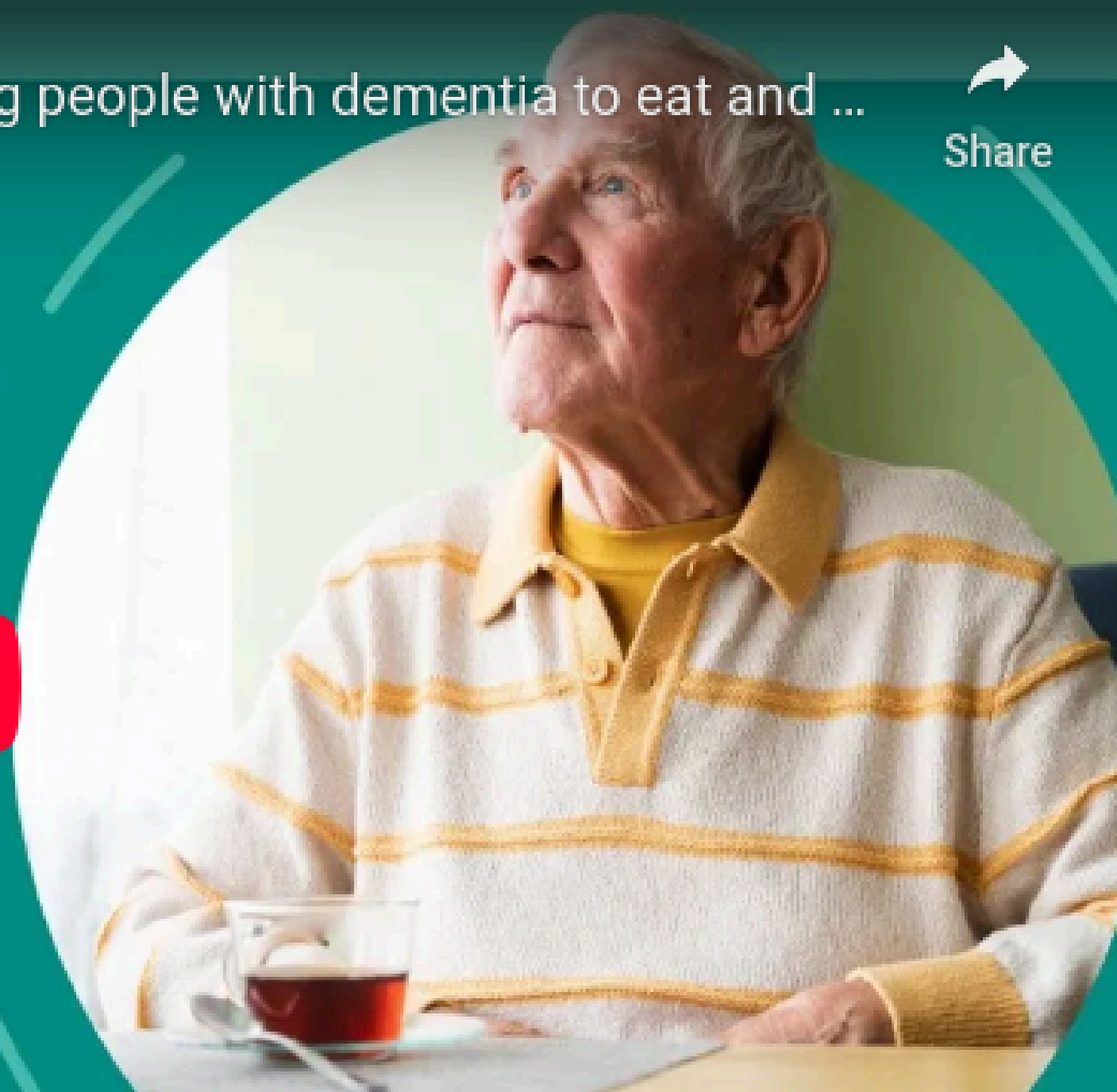
Julie Dundon
Advanced Accredited
Practising Dietitian,
member of the Aged
Care Quality and Safety
Advisory Council



Sue Hydes
Education Facilitator,
Centre for Dementia
Learning,
Dementia Australia.
Member of the
Commission's Food,
Nutrition and Dining
Expert Advisory Group



Danijela Hlis
Member of the
National Older
Person's Reference
Group, Older Persons
Advocacy Network
(OPAN)



Australian Government

Watch on



Quality and Safety Commission

agedcarequality.gov.au/food

**Food, nutrition
and dining hotline**

1800 844 044



Conclusion

Caring for individuals with dementia requires compassion, patience, and a tailored approach.

Each person's experience with dementia is unique, and understanding their personal triggers and needs is essential to providing effective care.



Knowledge Check:

Which of the following
is the most common
form of dementia?

A

Vascular Dementia

B

Alzheimer's Disease

C

Frontotemporal Dementia

D

Lewy Body Dementia



Knowledge Check:

What is the primary goal when managing Behavioural and Psychological Symptoms of Dementia (BPSD)?

A

To correct the behaviour through discipline.

B

To address the underlying causes and reduce distress.

C

To ignore the behaviour and let the individual calm down on their own.

D

To immediately administer medication to control symptoms.



Knowledge Check:

Which of the following is a non-confrontational communication strategy when caring for someone with dementia?

A

Speaking loudly and giving clear instructions.

B

Interrupting them when they talk.

C

Using a calm, slow voice and offering reassurance.

D

Arguing with the individual to ensure they understand.



Knowledge Check:

You should always stand up and look down at a person when communicating with them?

A

True

B

False

C

Unsure



Knowledge Check:

What does the ABC approach to behaviour management stand for?

A

Activity, Behaviour, Communication

B

Antecedent, Behaviour, Consequences

C

Approach, Behaviour, Consequences

D

Assess, Be Calm, Communicate



Answer Key

- **Question 1:** b) Alzheimer's Disease
- **Question 2:** b) To address the underlying causes and reduce distress
- **Question 3:** c) Using a calm, slow voice and offering reassurance
- **Question 4:** b) False
- **Question 5:** b) Antecedent, Behaviour, Consequences



References:



Dementia Australia: Anxiety

Available at: <https://www.dementia.org.au/living-dementia/mood-and-behaviour-changes/anxiety>

Dementia Australia: Changed behaviours

Available at: <https://www.dementia.org.au/living-dementia/mood-and-behaviour-changes>

Dementia Australia: What is dementia?

Available at: <https://www.dementia.org.au/about-dementia/what-is-dementia/>

Rural and remote practice

Available at: [https://www.collegianjournal.com/article/S1322-7696\(12\)00016-9/abstract](https://www.collegianjournal.com/article/S1322-7696(12)00016-9/abstract)

Mental Health Branch: Assessment and management of people with behavioural and psychological symptoms of dementia (BPSD)

Available at: <https://www.health.nsw.gov.au/mentalhealth/resources/Publications/assessment-mgmt-people-bpsd.pdf>

World Health Organisation: Dementia

Available at: <https://www.who.int/news-room/fact-sheets/detail/dementia>



**THANK
YOU**